

Severity & Case Classification

Task 4 — Classification of ADR Cases by Seriousness and Expectedness

Case 1 — Ciprofloxacin-Induced Urticaria

Patient	50-year-old male
Suspected Drug	Ciprofloxacin 500 mg orally BID
Indication	Bacterial infection
ADR	Generalized urticaria (hives)
Onset	Same day after starting treatment
Action Taken	Ciprofloxacin withdrawn; symptomatic treatment given
Outcome	Recovered

Classification

Seriousness	Non-serious
Expectedness	Expected

Seriousness rationale: Does not meet any of the 6 seriousness criteria — no death, no life-threatening event, no hospitalization or prolongation, no persistent disability, no congenital anomaly. The reaction was self-limiting and resolved fully after withdrawal and symptomatic treatment.

Expectedness rationale: Urticaria and other hypersensitivity skin reactions are well-documented, labeled adverse effects of fluoroquinolone antibiotics including ciprofloxacin.

Clinical impact: Low clinical impact — prompt recognition, drug withdrawal, and treatment led to full recovery with no lasting harm.

Case 2 — Penicillin-Induced Anaphylaxis

Patient	34-year-old female
Suspected Drug	Amoxicillin 500 mg orally
Indication	Upper respiratory tract infection
ADR	Anaphylactic reaction — facial swelling, dyspnea, hypotension
Onset	Within 30 minutes of first dose
Action Taken	Emergency treatment with IM epinephrine, IV fluids, ICU admission for 24 hours
Outcome	Recovered after ICU monitoring

Classification

Seriousness	Serious (Life-threatening; required hospitalization)
Expectedness	Expected

Seriousness rationale: Meets two seriousness criteria: the reaction was life-threatening (anaphylaxis with hypotension and respiratory compromise) and required hospitalization/ICU admission.

Expectedness rationale: Anaphylaxis is a well-recognized, labeled adverse reaction to penicillin-class antibiotics, though its severity in this instance was significant.

Clinical impact: High clinical impact — required emergency intervention; a serious reaction despite being a listed/expected event for this drug class.

Case 3 — Unexpected Hepatotoxicity with a Statin

Patient	62-year-old male
Suspected Drug	Atorvastatin 20 mg orally once daily
Indication	Hyperlipidemia
ADR	Acute liver injury with marked jaundice, not previously reported for this dose in this patient population
Onset	6 weeks after starting treatment
Action Taken	Drug discontinued; hepatology referral; liver function monitored
Outcome	Recovering, liver enzymes trending down

Classification

Seriousness	Serious (Medically significant / other important medical event)
Expectedness	Unexpected

Seriousness rationale: Classified as serious under the "other medically important condition" criterion, given the marked liver injury requiring specialist referral and close monitoring.

Expectedness rationale: While mild transaminase elevation is a known statin effect, marked jaundice/acute liver injury at this dose is more severe and specific than what is described in standard labeling, making this an unexpected reaction warranting expedited reporting.

Clinical impact: High clinical impact and reporting priority — unexpected + serious combination requires expedited regulatory reporting per most PV guidelines.

Case 4 — Mild GI Upset with Metformin

Patient	45-year-old female
Suspected Drug	Metformin 500 mg orally BID
Indication	Type 2 diabetes mellitus
ADR	Nausea and mild diarrhea
Onset	2 days after starting treatment
Action Taken	Dose taken with food; no discontinuation needed
Outcome	Resolved with continued therapy

Classification

Seriousness	Non-serious
Expectedness	Expected

Seriousness rationale: Mild, self-resolving symptoms; does not meet any seriousness criteria and did not require any medical intervention beyond a dosing adjustment.

Expectedness rationale: GI disturbances (nausea, diarrhea) are among the most common and well-labeled adverse effects of metformin, especially early in treatment.

Clinical impact: Minimal clinical impact — common, mild, and manageable without stopping therapy.